

Policy on Human Resource Planning and Governance

«Hospital Name»

Document control

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Prepared by	HR In-Charge / Personnel Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» plans its workforce so that the right number and mix of skilled staff is available now and in the future, not just when a crisis makes the gap obvious.

1. Purpose

This policy explains how «Hospital Name» plans staffing to meet patient care needs, defines jobs and reporting lines, checks new staff before they start, and learns from staff who leave.

This policy does not cover recruitment steps, induction training, or staff appraisal — those are covered in other hospital policies.

2. Scope

This policy applies to the HR In-Charge / Personnel Officer, department heads, and the Medical Superintendent, for every category of staff — full-time, part-time, employed, honorary, voluntary and temporary.

3. Policy standards

«Hospital Name» prepares an annual workforce plan against patient volume, services and technology in use; keeps enough staff, in the right mix, to meet patient needs; has a tested contingency plan for staff shortages; defines a job description for every staff category; background-checks new staff; documents reporting lines for every category of staff; and uses exit interviews to improve HR practice.

4. Non-negotiable rules

1. Do not let the workforce plan go a year without being compared against actual patient volume, services and staffing.
2. Do not leave a staffing shortfall unaddressed — trigger the contingency plan.
3. Do not let any staff category go without a defined job description and specification.
4. Complete the background check for every new staff member within one month of joining — never skip it.
5. Do not leave a staff role without a documented reporting line.
6. Do not let exit-interview findings sit unread — they must feed back into HR practice.
7. Staff who see any of these rules broken report it the same shift to the HR In-Charge / Personnel Officer or the Medical Superintendent.

5. What we do

5.1 Plan the workforce every year

The HR In-Charge / Personnel Officer prepares a workforce plan every year, comparing current staff numbers and skill mix against the hospital's mission, patient volume and mix, services offered, and medical technology in use, with input from department heads. Recognised staffing methods are used to set levels against the strategic and operational plan. Where the year's actual staffing varies from the plan, the corrective action taken is recorded and carried into the next year's plan.

5.2 Keep enough staff, in the right mix

«Hospital Name» keeps a number and mix of staff commensurate with workload and clinical need. Nursing staffing follows a published guideline (for example, WHO's Workload Indicators of Staffing Need method, or another recognised nursing staffing guideline). The HR In-Charge / Personnel Officer compares sanctioned against actual strength by department on a regular schedule and escalates an unresolved shortfall to the Medical Superintendent. Any shortfall triggers the contingency plan in 5.3.

5.3 Plan for staff shortages before they happen

The hospital keeps a written contingency plan for shift-by-shift, short-term and long-term workforce shortages, including unplanned ones. The plan may include reprioritising tasks, reallocating tasks across available staff, and drawing on a pool of filler staff such as previous employees or agency-sourced casual staff. Every shortage event is logged with its cause, the measure used, and the outcome. The plan itself is tested at a regular interval to confirm it still works.

5.4 Define every job

Every staff category at «Hospital Name» — full-time, part-time, employed, honorary, voluntary or temporary — has a job description and specification laying down the job's content and the qualifications, skills and experience needed. A role requiring the skills of a doctor or nurse carries a minimum qualification of MBBS or GNM respectively, unless a government or

statutory body has granted an exemption. The HR In-Charge / Personnel Officer holds the current set of job descriptions; a new hire signs to confirm they have received and understood theirs.

5.5 Check new staff before they start

«Hospital Name» performs a background check on every new staff member, using a defined method, either before the person joins or within one month of joining. The HR In-Charge / Personnel Officer keeps a register recording the method used, the date completed, and the outcome for each new hire. A staff member with no background check on file is escalated to the Medical Superintendent — this is not left for later.

5.6 Document who reports to whom

Reporting relationships are defined for every category of staff, documented as an organisation chart showing hierarchy, line of control and functions at each level. The chart is disseminated to all stakeholders, and reporting relationships are also defined at department or service level. The HR In-Charge / Personnel Officer holds the current chart.

5.7 Learn from staff who leave

«Hospital Name» conducts exit interviews and uses them to improve HR practice. A personal interview is the default method; taking part is voluntary for the departing staff member. The HR In-Charge / Personnel Officer compiles findings at a regular interval into a trend report for the Medical Superintendent and, where the hospital has one, the governing body, with proposed HR improvements tracked to closure.

6. Governance and responsibility

Role	Responsibility
HR In-Charge / Personnel Officer	Owns the workforce plan, staffing comparison, contingency plan, job descriptions, background checks, organisation chart, and exit-interview trend reports.
Department heads	Provide input to the workforce plan; confirm reporting lines and job content for their department.
Medical Superintendent	Accountable that this policy is followed; receives escalations on unresolved staffing shortfalls and exit-interview trend reports.

7. Quality monitoring

The HR In-Charge / Personnel Officer reviews a sample of records under this policy periodically against the steps in Section 5, confirming the workforce plan, contingency-plan tests, job descriptions, background checks, the organisation chart, and exit-interview trend reports are current. This policy itself is reviewed every year, and sooner if there is a major change in hospital services or staffing structure.

8. Training and staff acknowledgement

Staff covered by this policy — the HR In-Charge / Personnel Officer and department heads — are trained on this policy when they take up the role, and again every year after that. Training covers what's in Section 5 and the non-negotiable rules.

I have read the Policy on Human Resource Planning and Governance of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

HR In-Charge / Personnel Officer; department heads; Medical Superintendent; governing body, where applicable.

10. Abbreviations

Abbreviation	Meaning
GNM	General Nursing and Midwifery
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
HR	Human Resources
HRM	Human Resource Management (NABH Hospitals 6th Edition chapter)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element
MBBS	Bachelor of Medicine, Bachelor of Surgery

11. Traceability to NABH HCO Full Accreditation 6th Edition HRM.1

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
HRM.1.a	Commitment	Human resource planning supports the organisation's current and future ability to meet the care, treatment and service needs of the patient.	Section 3; 5.1	HR In-Charge / Personnel Officer
HRM.1.b	CORE*	The organisation maintains an adequate number and mix of staff to meet the care, treatment and	Section 3; 5.2	HR In-Charge / Personnel

		service needs of the patient.		Officer
HRM.1.c	Achievement	The organisation has contingency plans to manage long and short-term workforce shortages, including unplanned shortages.	Section 3; 5.3	HR In-Charge / Personnel Officer
HRM.1.d	Commitment	The job specification and job description are defined for each category of staff.	Section 3; 5.4	HR In-Charge / Personnel Officer
HRM.1.e	Commitment	The organisation performs a background check of new staff.	Section 3; 5.5	HR In-Charge / Personnel Officer
HRM.1.f	Commitment*	Reporting relationships are defined for each category of staff.	Section 3; 5.6	HR In-Charge / Personnel Officer
HRM.1.g	Achievement	Exit interviews are conducted and used as a tool to improve human resource practices.	Section 3; 5.7	HR In-Charge / Personnel Officer

12. Required Records / Evidence Checklist

HRM.1.a — Human resource planning supports current and future patient needs.

- Annual workforce plan comparing current and projected staffing against services and patient volume.
- Department-head input record into the plan.
- Corrective-action record for a variance found during the year.

HRM.1.b — Adequate number and mix of staff maintained.

- Sanctioned-versus-actual staffing comparison record by department.
- Staffing-norm reference used for nursing.
- Escalation record for an unresolved shortfall.

HRM.1.c — Contingency plans for workforce shortages.

- Written contingency plan for long- and short-term workforce shortages, including unplanned shortages.
- Shortage-event log with cause, measure used and outcome.
- Test record of the contingency plan.

HRM.1.d — Job specification and job description defined.

- Job description on file for each staff category, including qualification, skill and experience requirements.
- Signed acknowledgement record from a new hire receiving their job description.

- Minimum-qualification exemption record, where applicable.

HRM.1.e — Background check of new staff.

- Background-check register recording method, date and outcome per new hire.
- Completion record before or within one month of joining.
- Escalation record for any staff member with no background check on file.

HRM.1.f — Reporting relationships defined.

- Current organisation structure or chart showing hierarchy and reporting lines.
- Department- or service-level reporting-relationship record.
- Dissemination record to stakeholders.

HRM.1.g — Exit interviews conducted and used.

- Completed exit-interview record for a departing staff member.
- Trend report compiled from exit interviews.
- HR-improvement action record from exit-interview findings.

13. References

- NABH Accreditation Standards for Hospitals, 6th Edition — standard HRM.1.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — HRM.1 interpretations.
- World Health Organization. (n.d.). Workload Indicators of Staffing Need (WISN). <https://www.who.int/tools/wisn> — the staffing-norm method named in HRM.1's own interpretation as a good reference for determining nurse staffing levels.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

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